

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Patient: Patient 03 | Age: 55 years | Sex: Male

MRN: 000-TEST-03 | Admission Date: [admission date] | Discharge Date: [discharge date]

Facility: Metro Medical Center, Cardiology Service

****ADMITTING DIAGNOSIS & CLINICAL PRESENTATION****

Primary Dx: Congestive Heart Failure (CHF) with reduced ejection fraction (HFrEF)

Secondary Diagnoses:

- Type 2 Diabetes Mellitus (on metformin)
- Hyperlipidemia
- Hypertension

Patient presented to ED c/o progressive dyspnea on exertion (DOE) × 2 weeks, orthopnea, and lower extremity edema. Labs notable for elevated BNP 487 pg/mL (ref <100). Transthoracic echocardiogram revealed LVEF 32% consistent with Stage C HFrEF.

****HOSPITAL COURSE****

Pt admitted to telemetry unit for diuretic optimization and initiation of guideline-directed medical therapy (GDMT). IV furosemide infusion started at 40 mg BID; transitioned to oral dosing by hospital day 2 with clinical improvement. Euvolemia achieved by day 4 evidenced by resolution of orthopnea and reduced JVD. Daily weights trended downward (admission wt 94.2 kg → discharge wt 91.8 kg). SpO2 maintained 94–97% on RA. Patient tolerated PO intake without difficulty. No acute arrhythmias noted on continuous telemetry monitoring. Renal function stable; Cr 1.1 mg/dL throughout stay. Discharged in good condition after

patient education regarding sodium restriction, daily weight monitoring, and medication adherence.

****DISCHARGE MEDICATIONS****

Medication	Dose	Route	Frequency	Duration
Lisinopril (ACE-I)	10 mg	PO	q.d.	ongoing
Metoprolol Tartrate (BB)	25 mg	PO	BID	ongoing
Furosemide	40 mg	PO	q.d.	ongoing
Atorvastatin	40 mg	PO	q.d.	ongoing
Metformin	1000 mg	PO	BID	ongoing
Aspirin	81 mg	PO	q.d.	ongoing

****DIETARY RESTRICTIONS & FLUID MANAGEMENT****

- Sodium restriction: <2 g/day (1-2 tsp salt equivalent)
- Fluid restriction: 1.5–2 L/day
- Avoid processed foods, canned soups, deli meats
- Diabetic diet; coordinate with dietitian for carbohydrate counting

****ACTIVITY & WEIGHT-BEARING STATUS****

- Gradual increase in activity tolerance as tolerated; no strenuous exertion or heavy lifting (>10 lbs) × 4 weeks

- Walking 20–30 minutes daily encouraged
- Daily weights q.a.m. before breakfast; report gain >3 lbs in 24 hrs or >5 lbs in 1 week

****RED FLAG SYMPTOMS — SEEK IMMEDIATE CARE IF:****

- Acute dyspnea at rest or severe SOB
- Chest pain or pressure
- Syncope or severe dizziness
- Weight gain >5 lbs within 48 hours
- Persistent cough or hemoptysis
- Palpitations with lightheadedness

****FOLLOW-UP APPOINTMENTS****